

# Minison® Tablet

Prazosin Hydrochloride is an  $\alpha$ -adrenoreceptor blocking drug used in the treatment of hypertension. Chemically, it is designated 1-(4-Amino-6, 7-dimethoxy-2-quinazolinyl)-4-(2-furanylcarbonyl) piperazine hydrochloride.

## **Ingredient(s)**

Each tablet contains:

|                              |         |
|------------------------------|---------|
| Prazosin Hydrochloride ..... | 1.095mg |
| (eq. to Prazosin .....       | 1mg)    |

## **Pharmacodynamic:**

1. Prazosin, an antihypertensive agent, is thought to act by selective blockade of  $\alpha_1$  – adrenoreceptors producing peripheral dilatation of both arterioles and veins and reduction of peripheral resistance, usually without reflex tachycardia.
2. Prazosin reduces both preload and afterload and produces an improvement in cardiac output in patients with congestive heart failure.
3. Prazosin reduces both standing and supine blood pressure with a greater effect on the diastolic pressure.

## **Pharmacokinetics:**

Prazosin is readily absorbed from the gastrointestinal tract. It is highly bound to plasma protein. It is extensively metabolized in the liver and some of the metabolites are reported to have hypotensive activity. It is excreted as the metabolites and 5 to 11% as unchanged prazosin mainly in the feces. Less than 10% is excreted in the urine. It has a longer duration of action of up to 10 hours with a relatively short half-life of 2-3 hours.

## **Indication(s):**

Prazosin is indicated for the treatment of hypertension.

## **Dosage and Administration:**

Initial dose:

0.5mg (half tablet), 2-3 times daily. The initial dose should preferably be given before retiring to bed at night (to avoid postural hypotension syncope). Increase dose to 1mg, 2-3 times daily after 3-7 days.

Maintenance dose:

The therapeutic dosages most commonly employed have ranged from 6mg to 15mg daily given in divided doses. After initial titration some patients can be maintained adequately on a twice daily dosage regimen.

To be dispensed on physician's prescription.

## **Route of Administration:**

Oral.

## **Contraindication(s):**

Hypersensitivity to any component of this drug.

## **Precaution(s) / Warning(s):**

1. Treatment with Prazosin should be introduced cautiously because of the risk of sudden collapse following the initial dose.
2. Prazosin should be used with caution in patients with angina, and the dosage should be reduced in patients with renal failure or hepatic failure and in elderly.
3. Prazosin may cause drowsiness or dizziness; patients affected should not drive or operate machinery.
4. Safe use of this product during pregnancy and lactation has not been established. The product should be given to women who suspect pregnancy or to pregnant women and nursing mothers only when clearly needed. Prazosin has been shown to be excreted in small amount in human milk. Caution should be exercised when Prazosin is administered to nursing mothers.
5. Children: Not recommended for children under 12 years.

## **Interactions with Other Medicaments:**

Concomitant administration of diuretics and other antihypertensive agents may enhance the hypotensive effects of prazosin.

Chlorpromazine & amitriptyline:

Patient who was taking chlorpromazine and amitriptyline was reported to develop acute agitation on receiving Prazosin. The symptoms settled rapidly when Prazosin was discontinued.

Analgesics: Reduction of Prazosin-induced hypotension by indomethacin has been reported.

Calcium-channel blockers: Concurrent use of Verapamil and Prazosin has been reported to produce enhanced hypotensive effect in normotensive subjects. Markedly increased hypotensive responses have also been reported with combined use of Prazosin and Nifedipine, although the validity of such reports have been questioned.

Digoxin: Prazosin has been reported to increase the mean plasma-digoxin concentration in patients receiving a maintenance dose of digoxin.

## **Side Effect(s) / Adverse Reaction(s):**

Severe postural hypotension with syncope can occur following the initial dose of prazosin, and may be preceded by tachycardia. This reaction can be avoided by starting treatment with a low dose, preferably at night.

The most common side effects include dizziness, drowsiness, headache, lack of energy, nausea and palpitations, and may diminish with continued prazosin therapy or with a reduction in dosage.

Other side effects include postural hypotension, edema, chest pain, dyspnea, constipation, diarrhea, vomiting, depression, vertigo, hallucination, paresthesia, nasal congestion, dryness of mouth, urinary frequency, reddened sclera, blurred vision, tinnitus, skin rashes, pruritus, and diaphoresis.

**Symptoms and Treatment for Overdosage, and Antidote(s):**

Should overdosage lead to hypotension, support of the cardiovascular system is of first importance. Restoration of blood pressure and normalization of heart rate may be accomplished by keeping the patient in a supine position. If this measure is inadequate, shock should be first treated with volume expanders. If necessary, vasopressors should then be used. Renal function should be monitored and supported as needed. Laboratory data indicate Prazosin is not dialyzable because it is protein bound.

**Shelf- Life:**

3 years from the date of manufacture.

**Storage Condition(s):**

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

**Product Description & Packing(s):**

A light salmon color elliptical tablet, one side is impressed with a score.



Plastic bottle of 250's.

Plastic bottle of 500's and 1000's(for export only).

Blister packing of 10's x 50.



Manufacturer and Product Registration Holder:  
**Y.S.P. INDUSTRIES (M) SDN. BHD.** (199001001034)  
Lot 3, 5, 7, 12 & 14, Jalan P/7, Section 13,  
Kawasan Perindustrian Bandar Baru Bangi,  
43000 Kajang, Selangor, Malaysia.  
Ordering Line: 1 800 88 3027  
Product Info: 1 800 88 3679